

Transforming Certification for Better Care

# ABMS CONFERENCE 2026

## Meeting Physicians Where They Learn: Podcasting and AI in Certification Communication

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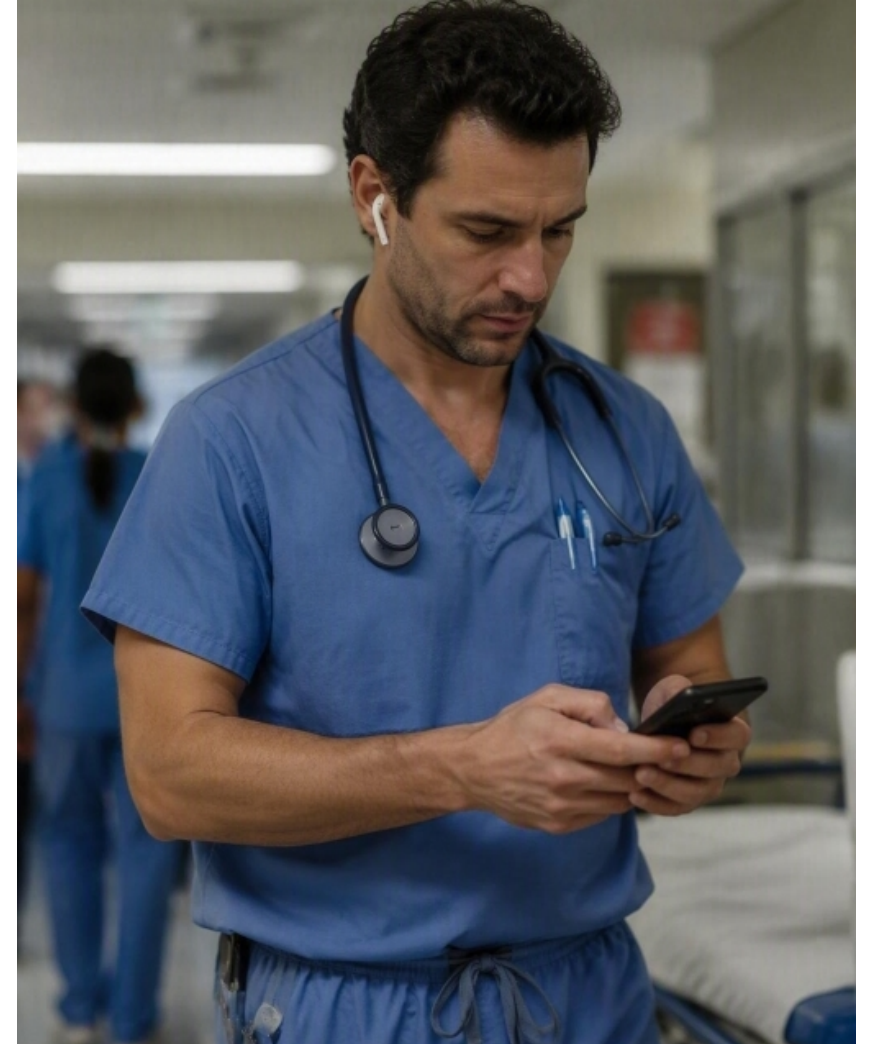
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# Disclosure

- No disclosures

# Why Podcast?

- Traditional channels are crowded: email blasts, newsletters, and portal notices compete for the same 30 seconds of attention
- Certification news is high-stakes but low-frequency — physicians look for it only when they need it
- Audio fits time already spent: commutes, workouts, pre-shift prep
- Voice carries tone, nuance, and credibility that a written notice cannot



# The Evidence: Audio Changes Practice

- Listeners rate podcasts 4.22 / 5 for impact on health knowledge — and 58% report changing health-related behaviors
- Why they listen: expert insights, enjoyment, and the ability to multitask
- 88.8% of Emergency Medicine Residents listen at least monthly; 72.2% say podcasts change their clinical practice
- Their top motivation: keeping up with current literature (88.5%) and learning core content (70.2%)
- What makes them pick a show: content relevance and presenter expertise rank

Sources: Barton et al., *Anat Sci Educ* 2025 (n=226); Riddell et al., *West J Emerg Med* 2017 (n=356).



# The Evidence: A Format That's Already Everywhere

- A scoping review of 62 studies found podcasts are a top resource for resident education, now entering formal curricula
- Reach extends beyond formal programs — a third of listeners surveyed were outside health training
- Orthopaedic podcasts grew 1,240% from 2016 to 2020 — about one new show a month
- Length is a design choice: listeners report preferring episodes of 30–60 minutes

*Sources: Kelly et al., Acad Med 2022; Barton et al., Anat Sci Educ 2025; Jella et al., JBJS Am 2021.*



# What Audio Does That Text Cannot

- Familiarity: a recurring voice becomes a trusted colleague, not an anonymous institutional sender
- Nuance travels: tone can convey “we hear you” on an exam change in a way an email never will
- Peers persuade peers: a diplomate’s own story lands differently than board guidance
- Passive reach: listeners consume while doing something else — no dedicated attention required
- **Panel prompt: which audiences were your traditional channels failing to reach?**

# How to Start a Podcast

- Audience and purpose: who are you speaking to, and what changes after they listen?
- Format and length: interview, panel, solo explainer, or short news drop
- Cadence you can sustain: reliable monthly beats an ambitious weekly that stalls
- Governance: who hosts, who reviews for accuracy, and who signs off

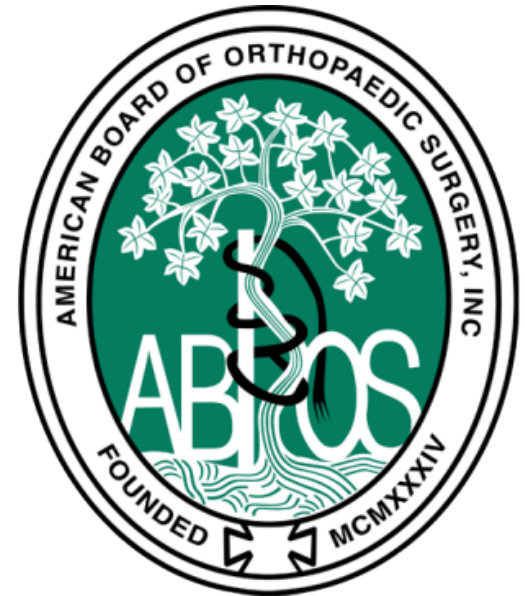
# Before You Hit Record: A Launch Checklist

- ✓ Determine audience and purpose
- ✓ Name one owner for scheduling, editing, and posting
- ✓ Agree on the review path: accuracy check, board sign-off, guest approval
- ✓ Purchase equipment and claim your host platform
- ✓ Define success: downloads, completion rate, fewer repeat questions to staff

Transforming Certification for Better Care

# ABMS CONFERENCE 2026

## Two Boards, Two Approaches



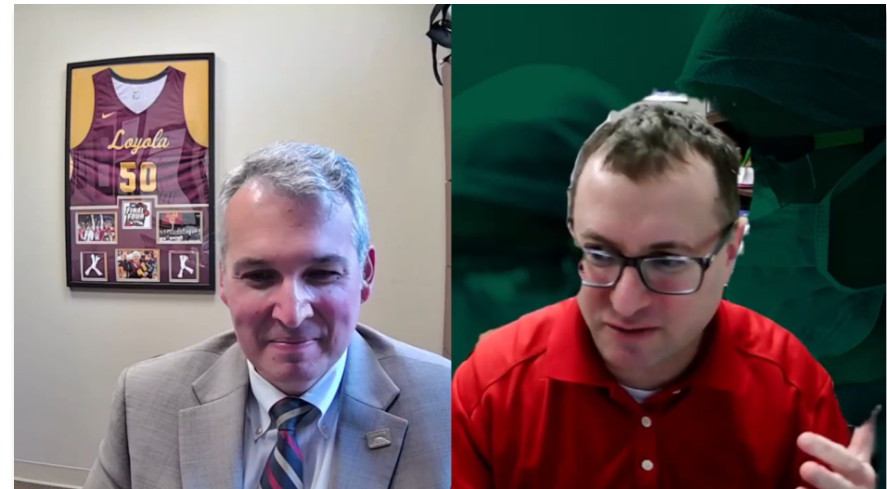
# ABOS Podcast

- Launched January 2020
- Most episodes can be broken down into one of these topics:
  - What is the ABOS?
    - Board Certification
    - Committee roles
  - Webinar replays
    - 5 annual podcasts, this is an audio-version
  - Examination tips from examinees (most popular)
  - Interesting people of the ABOS



# ABOS Podcast: Why Audio Is Different

- Writing is my first love, but audio is different
- We—the ABOS—will give general advice on passing an examination; an examinee is a step away and can give feedback that may not be appropriate for us—take the X study prep course
- Hearing from an ABOS legend is different than reading about them
- Speeding up a podcast may be more effective than speed reading



## Process: Planning the Episode

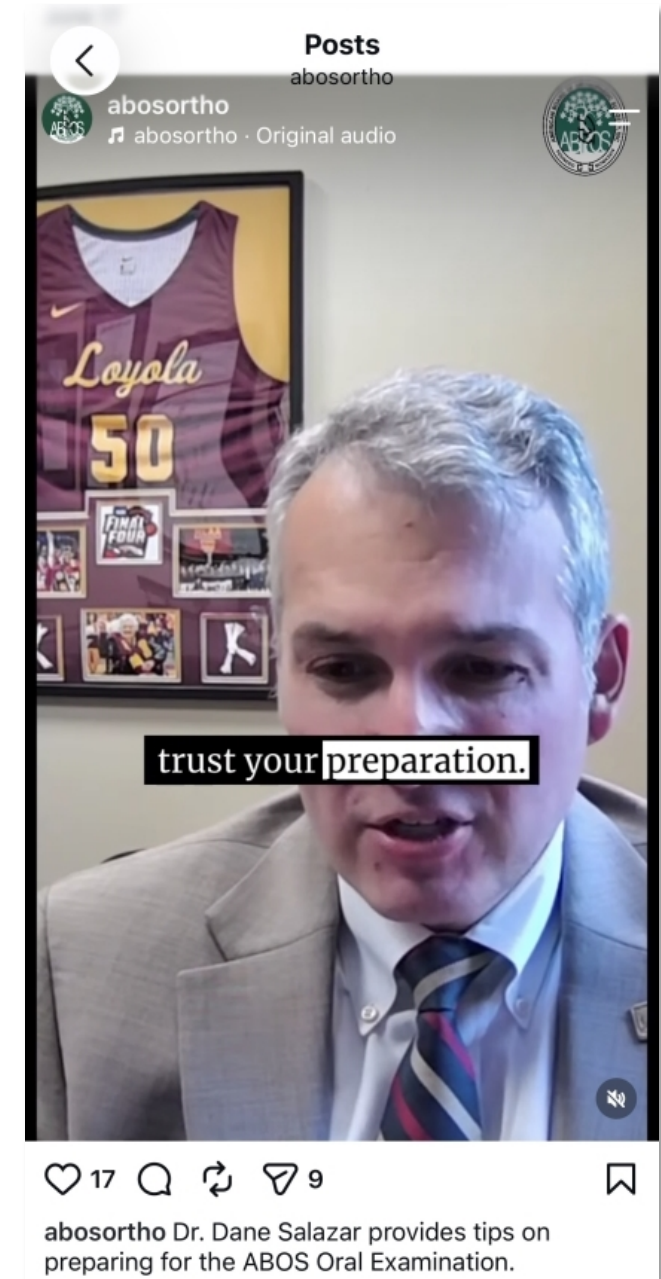
- Determine what the episode should be about
- Figure out who would be an appropriate guest—job title, knowledge, enthusiasm
- Invite them and discuss logistics
- Provide guest with questions—and sometimes answers
- Instruct them to review questions but do NOT write out answers—it will sound like they are reading it
- Prior to recording, I like talking to them for a bit so when it is actually time to start recording, they are "warmed up"

## Process: Recording and Editing

- It is not live—if they mess up, I will edit it out and nobody will know the difference
- I have messed up on 100% of the episodes—but nobody knows
- I generally do light editing (ums, you know)
- Occasionally, I have to make bigger edits
  - Incorrect information (that I miss live)
  - Foul language
  - Not the message we want to get out
    - This is not an unbiased podcast

# Process: Approval and Promotion

- Send the guest the edited version for approval
- Post
- Promote on social media
- Rinse, repeat



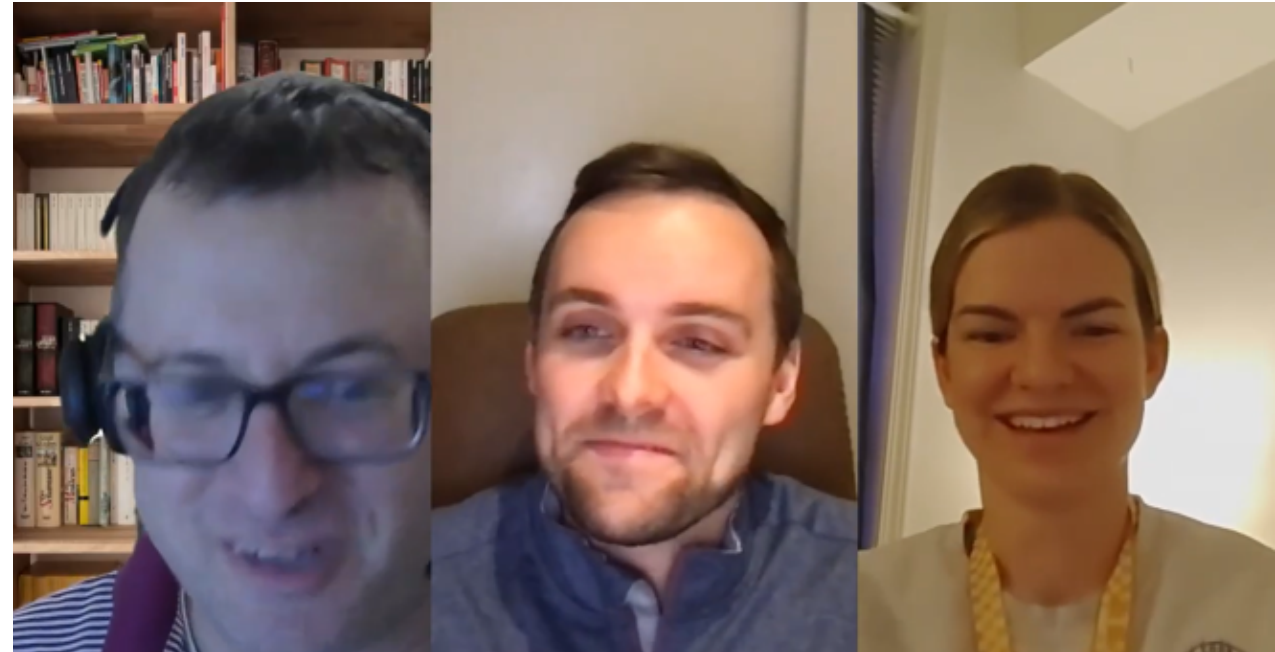
# Why I Do Podcasts

- In addition, not instead of, most communication
- Allows for extended conversations
- Slightly less formal method—it's a conversation, not an interview
- Shows our personality
- Makes us "look cool"
- Is fun



# Video Podcasts

- For nearly 2 years, YouTube has been the #1 app for enjoying podcasts
- ABOS entered video podcasts this year
- Need to tell guests that video—not just audio—is recording
  - Lighting
  - Hair/makeup
  - Background
- Slightly harder to edit video compared to audio-only
- YouTube Shorts, IG Reels, TikTok?!



# Equipment

- Riverside to record, edit, and distribute
- So easy to use that I don't need to ask a Millennial to help me
- Quiet place to record
  - Work, with door closed, turn off all other devices
  - Home, tell children (2- and 4-legged) to stay away
- Relatively inexpensive headphone and microphone



# ABEM GENERAL

## PODCAST



The *ABEM General* Podcast offers three different segments to keep you in the know. Listen on your favorite podcast app.



### ABEM Voices

Featuring interviews and conversations with ABEM-certified physicians across the country. Hear real-life stories of what the incredible work physicians are doing.



### ABEM Pathways

A focus on everything EM residents and those on the path to ABEM certification need to know.

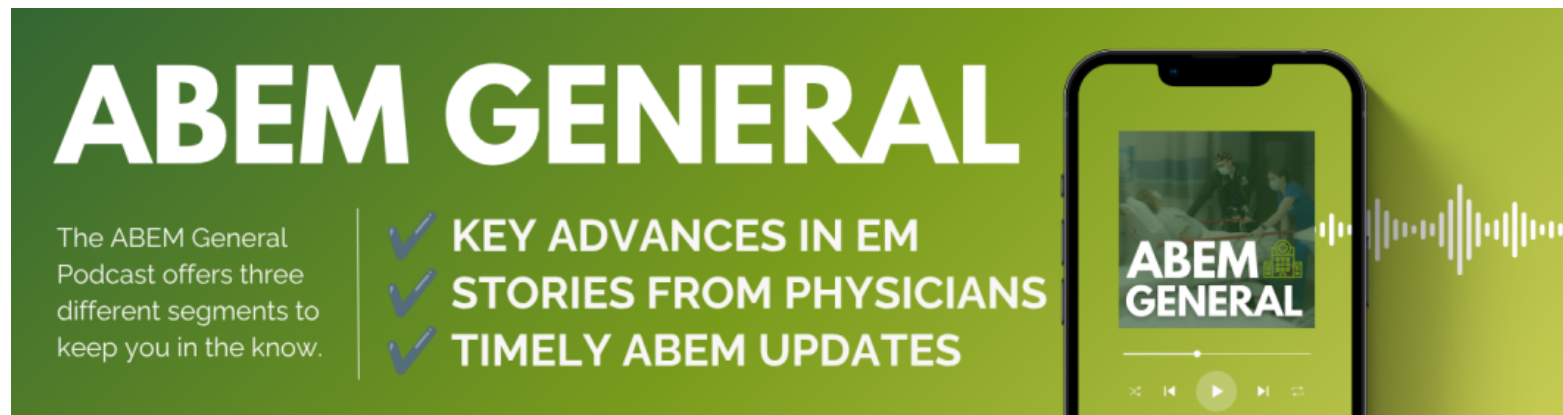


### ABEM Key Advances in EM

Stay up-to-date with the evolving practice of EM. Learn about practice advances, clinical policy alerts, and suggestions from literature that you can use to improve patient care.

# Development & Launch

- Researched what worked for others: advice from ABOS and inspiration from podcasts inside EM, across certification, and beyond both
- The ABEM Communications Committee approved the concept before production began
- Backlogged existing ABEM Voices interviews and webinar recordings so day-one listeners found a full library, not an empty feed
- Launched with three defined segments — Voices, Pathways, and Key Advances in EM — so every episode had a clear purpose and audience



# Workflow

**1** ABEM Leadership and Directors supply topic ideas and contributors



**2** ABEM President and Directors conduct the interview



**3** Remote recording — schedule Zoom and provide outline and questions in advance



**4** Use Canva to edit, add intro, outro, and ads



**5** Monthly review sessions to ensure consistent voice



**6** Publish monthly — a sustainable cadence



**7** Report analytics

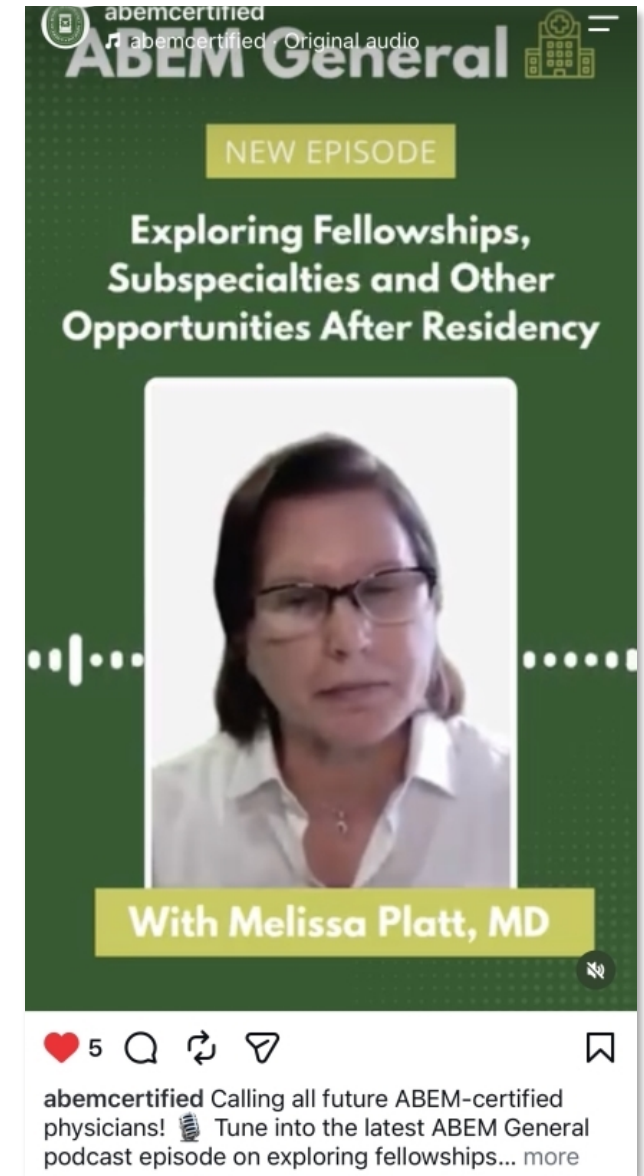
# Reusing Content

- One video recording can become many assets: a podcast with ads, a YouTube video, an interview-style news article, and short-form social clips
- The flow runs both directions — a social campaign can become a podcast, and a webinar recording can be published in multiple formats
- Repromote on relevance, not just recency: for Physician Suicide Prevention Day we reposted an ABEM Voices clip with an EBH volunteer who spoke on suicide and burnout
- Record the conversation once, then let it carry the message across multiple channels



# Ads: Promoting ABEM Inside ABEM

- Every episode creates an in-house ad opportunity for our own initiatives
- Placement is flexible: a spotlight at the top of an episode, or a reminder woven into the middle
- Example: a listener hearing an emergency physician's story also hears about MyEMCert modules or the HALM registration deadline
- Reaching diplomates who have already chosen to spend time with ABEM



# Listen In



# ABEM Key Advances in EM

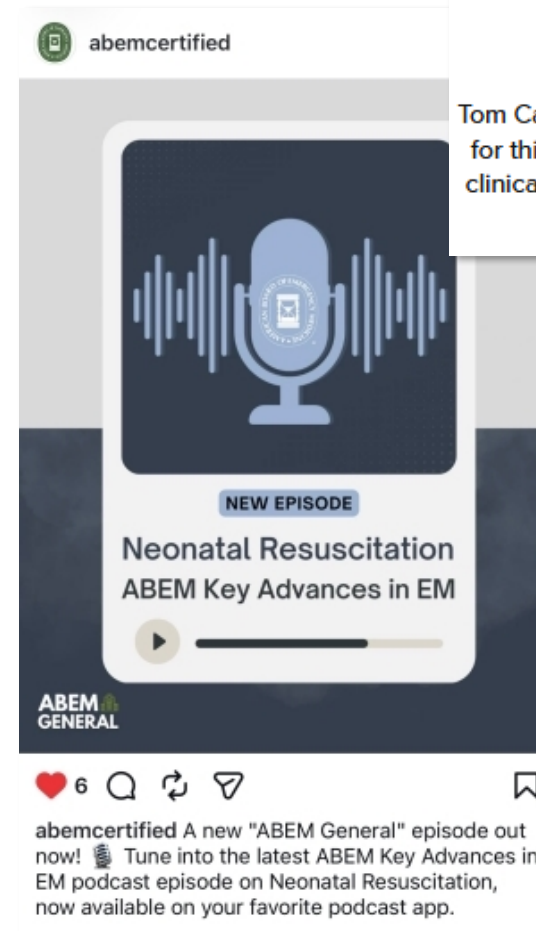
- Produced with AI using Jelly Pod — script, voice, and assembly
- The segment covers practice advances, clinical policy alerts, and literature suggestions: based on published Key Advances
- Reviewed for accuracy by practicing ABEM-certified physicians
- AI use is disclosed to listeners

## ABEM Key Advances in EM

Stay up-to-date with the evolving practice of EM. Learn about practice advances, clinical policy alerts, and suggestions from literature that you can use to improve patient care.



Tom Callahan and Harper Lane are your AI-generated hosts for this segment. All content is reviewed and validated by clinically active, board-certified emergency physicians and is based on **published key advances**.



# Listen In



KEY ADVANCES  
CLINICAL POLICY ALERT

## 2020 and 2024 Update on Neonatal Resuscitation

Updated July 2025

Aziz K, Lee HC, Escobedo MB, et al. Part 5: Neonatal Resuscitation: 2020 American Heart Association Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care. *Circulation*. 2020;142(16\_suppl\_2):S524-S550. doi:10.1161/CIR.0000000000000902

Sawyer T, McBride ME, Ades A, et al. Considerations on the Use of Neonatal and Pediatric Resuscitation Guidelines for Hospitalized Neonates and Infants: On Behalf of the American Heart Association Emergency Cardiovascular Care Committee and the American Academy of Pediatrics. *Pediatrics*. 2024;153(1):e2023064681. doi:10.1542/peds.2023-064681

Yamada NK, Szyld E, Strand ML, et al. 2023 American Heart Association and American Academy of Pediatrics Focused Update on Neonatal Resuscitation: An Update to the American Heart Association Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care. *Pediatrics*. 2024;153(2):e2023065030. doi:10.1542/peds.2023-065030

**Policy Recommendations and Focus Points in bold**

**Newborn resuscitation requires anticipation and preparation by providers who train individually and as teams.**

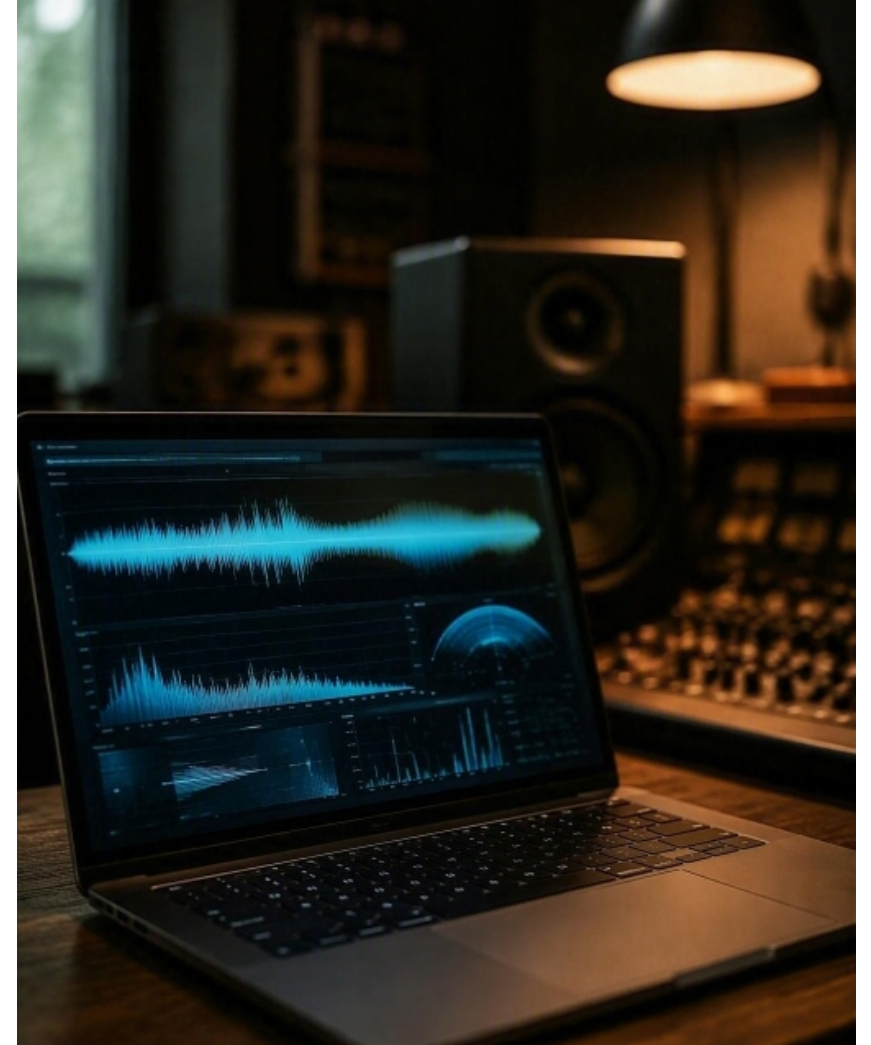
- Every birth should be attended by at least one person who can perform the initial steps of newborn resuscitation (i.e., dry, warm, and stimulate) and initiate positive pressure ventilation, and whose only responsibility is the care of the newborn. *Class of Recommendation 1, Strong*
- Before every birth, a standardized risk assessment tool should be used to assess perinatal risk and assemble a qualified team on the basis of risk. *Class of Recommendation 1, Strong*
- Before every birth, a standardized equipment checklist should be used to ensure the presence and function of supplies and equipment necessary for a complete

MyEMCert Key Advances Page 1



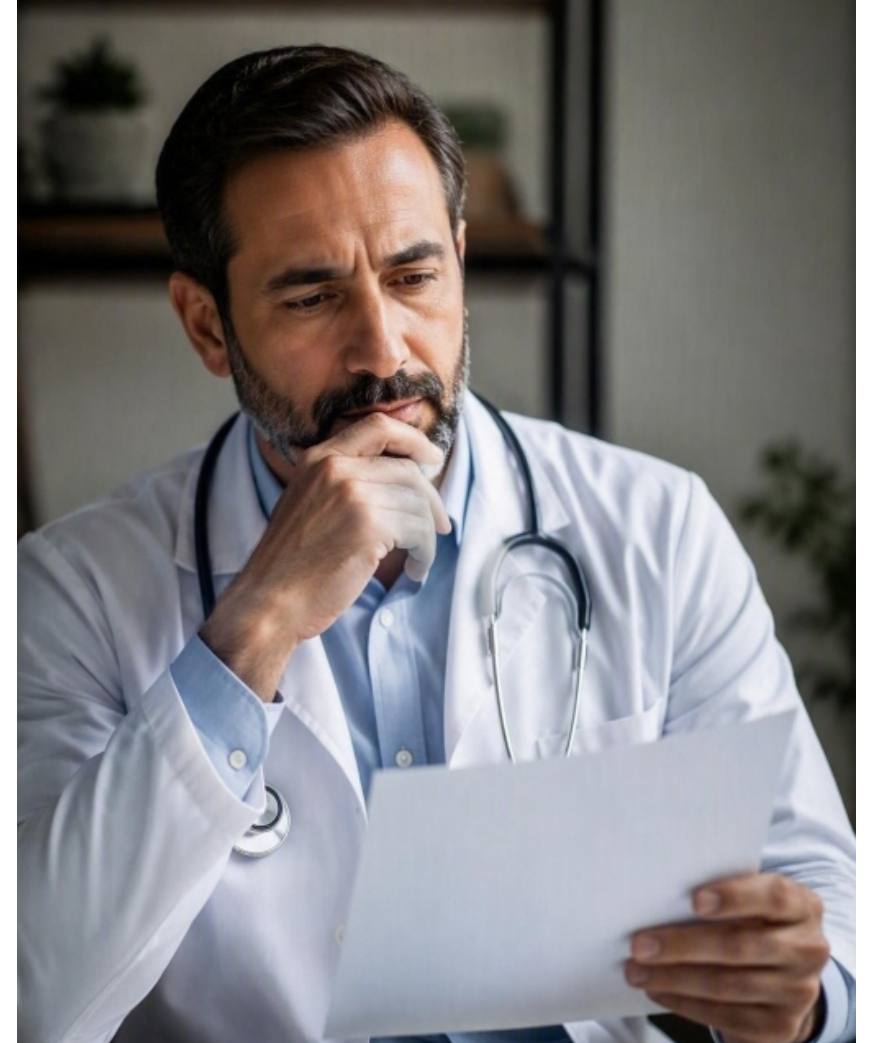
# AI in the Production Workflow

- Pre-production: topic research, question drafting, and episode outlines built from source material
- Production: noise reduction, design
- Post-production: transcripts, show notes, and social clips generated from the finished audio
- Rule of thumb: AI handles what repeats; humans handle what requires judgment



# Benefits, Limits, and Ethical Guardrails

- Benefits: faster turnaround, lower cost, steadier output, better accessibility
- Limits: confident errors, flattened clinical nuance, and a generic voice
- Ethics: disclose synthetic voices; protect board and diplomate data
- Accountability: AI saves hours but supplies no subject-matter judgment — validate information, especially medical advice



# Let's Discuss

- Discuss — name one channel you use to reach diplomates, and one place it falls short.
- Then go further: what is one topic your board is uniquely qualified to talk about for twenty minutes?
- Report out — offer one tactic worth stealing and one obstacle you are stuck on.
- Rapid poll — hands up: who is using AI in communications today? Where would you draw the line?

# Final Takeaways

- Audio meets physicians in time they already spend
- Start with audience, format, and a cadence you can sustain
- AI accelerates production but never replaces editorial judgment
- Questions?





Thank You